

## 2<sup>nd</sup> week Thursday case scenario

1-dr.

2-dr.

3-dr.

4- Dr.

### Case 41 ( Thyroid disease )

A 27-year-old lady who had one child of 2 years old who was delivered by emergency Caesarean section due to fetal distress, she came to you 2 days after her missing period and she performed pregnancy test (HCG) in urine and it was +ve. Her periods were regular(6/28)and she had H/O secondary hyperthyroidism due to small pituitary adenoma.

41.1 Dose she needs further investigation to confirm pregnancy?

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Later on after confirmed pregnancy she develops vaginal bleeding and her USS shows single viable fetus of 12 week and no abnormality

41.2 Do you think that corpus luteum insufficiency may be a cause of this bleeding and progesterone supplementation is needed?

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At 36 week of pregnancy she also develops mild vaginal bleeding and the USS shows anterior placenta with its lower margin 2-3 cm from internal os.

41.3 How you are going to deliver her and what is the significant risk in this patient during and after delivery and why?

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41.4 what are the complications of secondary hyperthyroidism if un treated?

How can hyperthyroidism affect me and my baby?